

1 — PATIENT INFORMATION

Case label / initials: _____

Date: _____

MRP / Attending: _____

Time code called: _____

Nurse: _____

Pt weight (kg): _____

2 — ONSET TIME

☐ **Known onset** — time: _____ ☐ **Unknown onset** ☐ **Wake-up stroke**

Last Known Normal (LKN): _____ → Hours from LKN to now: _____ h

☒ <4.5 h from LKN	TNK window OPEN
☐ 4.5 – 24 h from LKN	TNK closed · EVT may be possible if LVO present
☐ >24 h	Outside standard treatment windows

3 — ABSOLUTE CONTRAINDICATIONS TO TNK

Any YES = absolute contraindication to TNK (can still assess for EVT)

Y N ? Item

- ☐ ☐ ☐ Active hemorrhage / high bleeding risk
- ☐ ☐ ☐ BP refractory — cannot maintain <185/110 despite IV antihypertensives
- ☐ ☐ ☐ Blood glucose <2.7 mmol/L (treat hypoglycemia first)
- ☐ ☐ ☐ Blood glucose >22.2 mmol/L
- ☐ ☐ ☐ Platelet count <100,000/mm³

Y N ? Item

- ☐ ☐ ☐ INR >1.7
- ☐ ☐ ☐ aPTT elevated (above normal)
- ☐ ☐ ☐ DOAC within 48 hours (dabigatran, rivaroxaban, apixaban, edoxaban)
- ☐ ☐ ☐ Therapeutic LMWH in past 24 h (VTE prophylaxis dose is OK)
- ☐ ☐ ☐ Hemorrhage on CT / brain imaging

4 — RELATIVE CONTRAINDICATIONS / HISTORY

Require clinical judgment — YES items do not automatically prevent TNK but must be discussed

Y N ? Item

- ☐ ☐ ☐ History of intracranial hemorrhage
- ☐ ☐ ☐ Stroke or serious head/spinal trauma in past 3 months
- ☐ ☐ ☐ MI or pericarditis in past 3 months
- ☐ ☐ ☐ Major surgery in past 14 days
- ☐ ☐ ☐ Arterial puncture at non-compressible site in past 7 days
- ☐ ☐ ☐ GI or GU bleed in past 21 days
- ☐ ☐ ☐ Seizure at onset of stroke

Y N ? Item

- ☐ ☐ ☐ Decreased LOC at onset
- ☐ ☐ ☐ Pregnant
- ☐ ☐ ☐ Currently on warfarin (non-DOAC) — INR must be <1.7
- ☐ ☐ ☐ Severe pre-existing disability
- ☐ ☐ ☐ Age >80 + prior stroke + diabetes (in 3–4.5 h window)
- ☐ ☐ ☐ Onset 3–4.5 hours ago (increased risk in this window)

Clinical notes:

5 — NIHSS SCORE SHEET (circle or write score in the Score box)

#	Item	Score Descriptors — circle the best fit	Score (write here)
1a	Level of Consciousness	0 Alert, responsive 1 Drowsy, arousable by voice 2 Obtunded, needs repeated stimulation 3 Unresponsive/reflex only	max 3
1b	LOC Questions "Month?" "Age?"	0 Both correct 1 One correct (or intubated/severe dysarthria) 2 Neither correct / unable by any means	max 2
1c	LOC Commands Close eyes / Fist	0 Both correct 1 One correct 2 Neither	max 2
2	Best Gaze Horizontal eye mvt	0 Normal — iris reaches medial canthus both sides 1 Partial gaze palsy — incomplete, no forced deviation 2 Forced deviation — cannot overcome	max 2
3	Visual Fields Confrontation / blink to threat	0 No loss 1 Partial hemianopia (1 quadrant) 2 Complete hemianopia 3 Bilateral (cortical blindness)	max 3
4	Facial Palsy Show teeth / eyebrows / close eyes	0 Normal — symmetric 1 Minor lower face asymmetry 2 Complete lower face paralysis (upper spared) 3 Complete — upper + lower face (peripheral pattern)	max 3
5a	Right Arm Motor Seated 90° or supine 45° — 10 sec	0 Holds 10 s — no drift 1 Drifts, no bed contact 2 Effort vs gravity, touches bed 3 Falls immediately, moves on surface 4 No movement UN Untestable	max 4
5b	Left Arm Motor Same as 5a	0 Holds 10 s 1 Drifts, no bed contact 2 Effort vs gravity, touches bed 3 Falls, moves on surface 4 No movement UN Untestable	max 4
6a	Right Leg Motor Supine 30° — 5 sec	0 Holds 5 s — no drift 1 Drifts, no bed contact 2 Effort vs gravity, touches bed 3 Falls immediately, moves on surface 4 No movement UN Untestable	max 4
6b	Left Leg Motor Same as 6a	0 Holds 5 s 1 Drifts, no bed contact 2 Effort vs gravity, touches bed 3 Falls, moves on surface 4 No movement UN Untestable	max 4
7	Limb Ataxia Finger-to-nose / Heel-to-shin	0 Absent (score 0 if weakness prevents testing) 1 Ataxia in one limb 2 Ataxia in two or more limbs	max 2
8	Sensory Pin prick — face, arms, legs (not hands/feet)	0 Normal — equal bilaterally 1 Mild–mod loss — aware of touch, feels different 2 Severe loss — does not feel touch on affected side	max 2
9	Best Language Cookie jar picture / naming / reading	0 No aphasia — normal fluency, naming, comprehension 1 Mild–mod — word-finding pauses, gets ideas across 2 Severe — fragmented, examiner must guess meaning 3 Mute / global — no usable speech, no comprehension	max 3
10	Dysarthria "Mama" "Tip-top" "Huckleberry"	0 Normal articulation 1 Mild–mod slurring — still intelligible 2 Severe — unintelligible or mute UN Intubated	max 2
11	Extinction & Inattention Bilateral simultaneous stimulation	0 No abnormality 1 Inattention in 1 modality (visual OR sensory) 2 Profound neglect — >1 modality, may not recognize own hand	max 2

NIHSS TOTAL: _____ / 42

0 = no symptoms · 1–4 = minor · 5–15 = moderate · 16–20 = moderate–severe · ≥21 = severe

6 — CT / IMAGING RESULTS

CT Head:

☐ Clear of hemorrhage

☐ Hemorrhage present (→ TNK contraindicated)

☐ Pending

ASPECTS (0–10, ≥6 needed for TNK): _____

CTA — Vessel Imaging:

☐ Done

☐ Pending / Not done

LVO: ☐ Yes ☐ No

Vessel: ☐ ICA-T ☐ M1 ☐ M2 ☐ ACA ☐ Basilar ☐ Other: _____

7 — TNK DECISION**TNK Eligibility:**

- ☐ ☒ **TNK INDICATED** — No absolute CIs, CT clear, within window
- ☐ ☒ **TNK CONTRAINDICATED** — Absolute CI present (specify): _____
- ☐ ☒ **Clinical discussion required** — Relative CIs / borderline case

EVT (Endovascular Therapy):

- ☐ EVT referral made — Time: _____
- ☐ Not indicated / pending discussion
- Transfer destination: _____

8 — TNK DOSING 0.25 mg/kg IV bolus — Maximum 25 mg

Weight (kg)	Dose (mg)	Weight (kg)	Dose (mg)	Weight (kg)	Dose (mg)
40	10.0	62	15.5	84	21.0
44	11.0	66	16.5	88	22.0
48	12.0	70	17.5	92	23.0
52	13.0	74	18.5	96	24.0
56	14.0	78	19.5	100	25.0 ★
60	15.0	82	20.5	★ Max dose 25 mg	

Patient weight: _____ kg
 Calculated dose: _____ mg
 Formula: weight × 0.25, capped at 25 mg

9 — CONSENT & ADMINISTRATION

Consent obtained with: ☐ Patient ☐ SDM

SDM name (if applicable): _____

☐ ☒ **TNK Given** — Time: _____

☐ ☒ **TNK Not Given**

Reason not given: _____

BP Pre-TNK (target <185/110 mmHg):

BP: _____ Time: _____

BP med used:

☐ Labetalol 10 mg IV ☐ Hydralazine 10 mg IV ☐ Nicardipine infusion

Post-TNK monitoring:

BP q15 min × 2 h → q30 min × 6 h → q1 h × 16 h

Target BP <180/105 for 24 h post-TNK

Watch for: headache, neuro deterioration, BP spike, bleeding

NOTES